

Patient: [Redacted] Referring Physician: [Redacted]

DOE Age: Gender:

Ref#: Hosp#: Provider Group:

Date of Service: Date Received: Outpatient Room:

UID:1DA19725-B6C2-4487-A164-2FAB6CD3DE2C
TCGA-FY-A3YR-01A-PR Redacted



Case #: Date Reported:

FINAL SURGICAL PATHOLOGY REPORT

Diagnosis:

A. &B.) THYROID GLAND, RIGHT AND LEFT LOBES, TOTAL THYROIDECTOMY:

- PAPILLARY THYROID CARCINOMA.
 - 1.7 cm in greatest dimension.
- TUMOR INVOLVES PERITHYROIDAL ADIPOSE TISSUE.
- MARGIN STATUS: NEGATIVE.
- THREE OF FIVE LYMPH NODES, POSITIVE FOR METASTASIS (3/5).
 - Extranodal extension: Not identified.
- MULTIPLE ADENOMATOID NODULES (MULTINODULAR GOITER).
- SEE TUMOR STAGING SUMMARY BELOW.

C. CENTRAL NECK LYMPH NODE DISSECTION:

- ONE OF SIX LYMPH NODES, POSITIVE FOR METASTATIC CARCINOMA (1/6).
 - Extranodal extension: Not identified.
- ONE BENIGN PARATHYROID GLAND.

ICD03
Carcinoma, papillary, thyroid
8260/3

Sit: thyroid, NOS
C73.9

6-6-12 RD

PATHOLOGIC TUMOR STAGING SUMMARY:

Tumor type: Papillary thyroid carcinoma, classical variant.
Primary tumor: pT3 (1.7 cm in greatest dimension, with minimal extrathyroidal extension).

Regional lymph nodes: pN1a (4 of 11 total lymph nodes, positive for metastatic carcinoma; 4/11).

Distant metastasis: Not applicable.

Pathologic stage: III

Lymphovascular invasion: Not identified.

Margin status: R0.

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case Rejected		
Reviewer Initials		
Date Reviewed		

QUALIFIED DISQUALIFIED
5/31/12

COMMENT: The patient's previous thyroid fine needle aspiration cytology result of papillary thyroid carcinoma is noted and correlates with the current specimen.

Tumor Staging Information

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Data derived from current specimen. Staging in accordance with or modified from AJCC Cancer Staging Handbook, and CAP protocol,

Procedure:

Total thyroidectomy with central compartment dissection.

Specimen type:

Thyroid gland, and central compartment.

Specimen integrity:

Intact.

Specimen size:

Right lobe: 2.7 x 2.8 x 1.5 cm.

Left lobe: 3.8 x 2.2 x 1.7 cm.

Central compartment: 2.5 x 2.3 x 0.7 cm.

TUMOR FEATURES:

Tumor size:

Greatest dimension: 1.7 cm.

Additional dimensions: 1.3 x 1.2 cm.

Tumor focality:

Unifocal.

Tumor laterality:

Right.

Histologic type:

Papillary thyroid carcinoma.

Lymphovascular invasion:

Not identified.

Tumor capsule:

Unencapsulated.

Tumor capsular invasion:

Not applicable.

Extrathyroidal extension:

Minimal, extending into perithyroidal adipose tissue.

LYMPH NODES:

Eleven total lymph nodes; four positive for metastatic carcinoma.

MARGIN EVALUATION:

Distance to closest margin:

Negative (R0).

Other margins:

0.15 mm from anterior margin, at superior pole.

Uninvolved by carcinoma.

PATHOLOGIC TUMOR STAGING DESCRIPTORS:

Primary tumor:

pT3 (1.7 cm in maximum dimension, with extension into perithyroidal adipose tissue).

Regional lymph nodes:

pN1a (4/11 lymph nodes, positive for metastasis).

Distant metastasis:

Not applicable.

Margin status:

R0.

Pathologic stage:

III.

Additional pathologic findings:

Multiple adenomatoid nodules (multinodular goiter).

Comment:

Not applicable.

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Signed

Source of Specimen:

- A. Thyroid; Right thyroid lobe
- B. Thyroid; left lobe thyroid
- C. Thyroid; central node (thyroid)

Clinical History/Operative Dx:

Thyroid cancer

Gross Description:

A. Single specimen designated as right thyroid lobe. Initially received in the fresh state for Oncogenotyping studies is a 6 gram lobe of thyroid, 2.7 x 2.6 x 1.5 cm. The surface of the thyroid is partially glistening, lobular, deep red with adhesions. The posterior and anterior margins are now differentially inked black and blue, respectively. The specimen is serially sectioned perpendicularly sectioned through the medial-lateral long axis to reveal a fairly circumscribed, dense, pink-gray nodular lesion abutting the superior pole and the anterior and posterior margins, measuring 1.7 x 1.3 x 1.2 cm. Representative portions of tumor are retrieved centrally from the mass and submitted for Oncogenotyping studies. Notably, the lesion does not appear to have a capsular margin. Examination of the remaining cut sections demonstrates meaty deep red-tan parenchyma with a 0.9 x 0.8 x 0.7 cm gelatinous nodule near the inferior pole. The lobe of thyroid is entirely submitted for microscopic evaluation in a sequential fashion, superior to inferior in A1-A6.

B. Part B designated left lobe of thyroid. Received in formalin is a 6 gram lobe of thyroid, 3.8 x 2.2 x 1.7 cm. Attached to the lobe of thyroid is a 2.7 x 1.8 x 0.3 cm pad of fat containing three lymph node candidates ranging from 0.2 cm to 0.6 cm each. The lobe of thyroid demonstrates a glistening lobular surface with moderate adhesions. The posterior and anterior margins are now differentially inked black and blue, respectively. The lobe is serially sectioned perpendicular through the superior-inferior long axis revealing a predominantly deep red meaty thyroid parenchyma, with a single discrete pink-gray nodule, 0.3 x 0.3 x 0.2 cm, within 0.1 cm of the anterior thyroid margin, within 1.2 cm of the superior apex (B4). The thyroid is entirely submitted for microscopic evaluation in sequential fashion superior to inferior, including entire submission of the lymph node tissue candidacy and remaining fat.

Cassette summary: B1) small lymph node candidate and residual fat submitted, B2) two lymph node candidates, B3-B8) thyroid lobe entirely submitted superior to inferior, lesion demonstrated in B4.

C. Part C designated as central node thyroid. Received in formalin are multiple fragments of yellow-tan

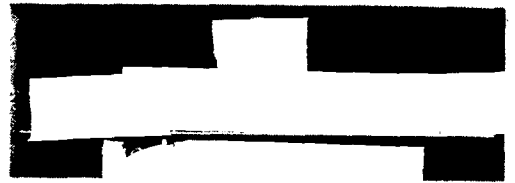
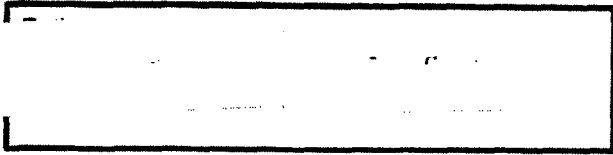
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lobular fat, measuring in aggregate of 2.5 x 2.3 x 0.7 cm. Examination reveals seven lymph node candidates ranging from 0.2 cm to 0.6 cm, rubbery to firm pink and tan. C1) three lymph node candidates, C2) four lymph node candidates.

Microscopic Description:

A. Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

B. Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

C. Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

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